1. The headline of the article. The author. The publication data.

The article I'm going to give a review of is taken from the website for psychology articles (verywellmind.com.).

The headline of the article is "Symptoms and Diagnosis of PTSD".

It is written by PhD Matthew Tull.

The article is written on the 20th of April in 2021.

2. The topic of the article and logical parts.

The topic of the article is post-traumatic stress disorder.

The purpose of the article is to give the reader some information on criteria for PTSD and PTSD diagnosis.

The article under discussion may be divided into several logically connected parts which are the introduction, the main part and the conclusion. The introduction gives general information about PTSD and the difference between this disorder and stress. Criteria for PTSD and diagnosis are described in the main part. In conclusion the author tells us about the related conditions that can be experienced after a traumatic event.

3. The content of the article. Summary.

The author starts by telling the reader that PTSD is caused by traumatic event.

The author draws reader's attention to the fact that if a person experiences symptoms of PTSD he should have professional help. It is also important to draw a distinction between PTSD and stress.

Much attention is given to criteria for PTSD.

According to DSM there are several criteria for PTSD which are called criterion A – stressor, criterion B – intrusion symptoms, criterion C – avoidance, criterion D – negative alterations in mood, criterion E – alterations in arousal and reactivity, criterion F – duration, criterion G – functional significance, criterion H – exclusion.

The article goes on to say that stressor includes exposure or threat of death, serious injury, or sexual violence which can be experienced by a person, or a person witnesses the event happen to someone else, learns of a close relative or friend who experienced such event(s), faces to the event(s) in the course of professional duties. It is reported that there are intrusion symptoms of PTSD such as recurrent, involuntary, and intrusive memories; traumatic nightmares or upsetting dreams with content related to the event; dissociative reactions, such as flashbacks, in which it feels like the experience is happening again; intense or prolonged distress after exposure to traumatic reminders; marked physiological reactivity, such as increased heart rate, after exposure to traumatic reminders. Criterion C, which is avoidance, includes persistent effortful avoidance of distressing trauma-related reminders (related thoughts or feelings, or

related external reminders, such as people, places, conversations, activities, objects, or situations) after the event. Negative alterations in cognition and mood that began or worsened after the traumatic event, e.g., ersistent, and often distorted negative beliefs and expectations, persistent negative emotions, including fear, horror, anger, guilt, or shame etc., belong to criterion D. Trauma-related alterations in arousal and reactivity that began or worsened after the traumatic event, e.g., irritable or aggressive behavior, self-destructive or reckless behavior, present criterion E. Criterion F is persistence of symptoms in Criteria B, C, D, and E for more than one month. Criterion G includes significant symptom-related distress or impairment of different areas of life, such as social or occupational. Criterion H represents the disturbance is not due to medication, substance use, or other illness.

It is spoken in detail about PTSD diagnosis. In order to be diagnosed with PTSD according to the DSM-5, a person needs to meet the following: criterion A, one symptom or more from criterion B, one symptom or more from criterion C, two symptoms or more from criterion D, two symptoms or more from criterion E, criterion F, criterion G, criterion H. The fact that there are a few changes in the latest version of the DSM regarding PTSD diagnosis is stressed. Key changes are more clearly defining what kind of events are considered traumatic in criterion A; adding a fourth type of exposure in criterion A; increasing the number of symptom groups from three to four by separating avoidance symptoms into their own group (criterion C); increasing the number of symptoms from 17 to 20; changing the wording of some of the symptoms from DSM-IV; adding a new set of criteria for children aged 6 or younger; eliminating the "acute" and "chronic" specifiers; introducing a new specifier "with dissociative symptoms".

Further the author draws the reader's attention to that there are other diagnostic tools such as physical examination and psychological evaluation.

The author comes to the conclusion that there are a variety of distressing symptoms that someone with PTSD will experience after the event such as obsessive-compulsive disorder, acute stress disorder, panic disorder etc. It is important to talk with a qualified professional to make sure you receive an accurate diagnosis because some of the distressing symptoms can overlap a bit with other mental health conditions.

4. Your opinion (conclusion)

Taking into consideration the fact that PTSD is a common disorder and must be corrected this article is currently important.

The main idea of the article is describing criteria for PTSD which is crucial in diagnosing of PTSD.

In addition, some facts about PTSD diagnosis are given.

Back to our main topic it is also important to draw a distinction between PTSD and other mental disorders.

From my point of view the article is easy to understand. The information is well-structured and well-ordered. The vocabulary is vivid and has a lot of terms.

I fully agree with the fact that a person should have professional help if he is experiencing the symptoms of PTSD, and the diagnosis must be accurate in order to be properly treated.

I have found the article of great value because PTSD is a serious mental illness. It is not acceptable to self-medicate because it can lead to after-effects.